Public Health Scotland

General Practice Workforce Survey 2022

An Official Statistics release for Scotland (Experimental)

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This is an Experimental Statistics publication

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Contents

Introduction 5

Main points 7

Results and commentary 8

General Practitioners 8

Headcount and estimated Whole Time Equivalents (WTEs) 8

Absences and Use of locum/sessional GP time 14

Reported GP vacancies 17

Nurses, Health Care Assistants, Phlebotomists and Other Staff 19

Nurses 19

Health Care Assistants and Phlebotomists 24

Other Staff 26

Glossary 27

Contact 30

Further information 30

Open data 30

Rate this publication 30

Appendices 31

Appendix 1 – Background information 31

Appendix 2 – Methodology 32

Data 32

GP WTE Estimates 33

Nurse headcount and WTE Estimates 34

Appendix 3 – Publication metadata 35

Appendix 4 – Early access details 39

Pre-Release Access 39

Standard Pre-Release Access: 39

Early Access for Management Information 39

Early Access for Quality Assurance 39
Appendix 5 – PHS and Official Statistics 40
About Public Health Scotland (PHS) 40

Introduction

This release by Public Health Scotland provides information on estimated Whole Time Equivalent (WTE) of General Medical Practitioners (GPs) in Scotland up to 2022, broken down by age and sex, GP designation and NHS Board. It also provides estimates of headcount and WTE for nurses at general practices in Scotland.

The **National Primary Care Workforce Survey** was last carried out in 2017. It was replaced in 2019 by a **GP Data Collection Tool**, and the first results were published in the **General practice workforce survey** in 2021. The **GP Data Collection Tool** was used again to collect data on the GP, nurse and other professional workforce in general practice in 2022. Information on the Primary Care out of hours workforce will be published separately in early 2023.

This report summarises the results from the workforce element of the 2022 survey. A full list of charts and tables are available to view (and download to Excel) in the accompanying **Interactive Dashboard.**

Information up to 2021 on GP headcount in Scotland, by NHS Board, GP designation, age and sex is already available to view in the annual **GP Workforce And Practice List Sizes** National Statistics publication. This will be updated to include data for 2022 in December 2022.

Due to the new method of collecting data in 2019, quality assurance was carried out by PHS to ensure reliability of the data. Figures for GP WTE, nurse WTE and nurse headcount are all estimates.

GP WTE figures have been scaled to correspond to GP headcount figures derived from the National Primary Care Clinicians Database (NPCCD), which is maintained by NHS Boards and is the source database for the **GP Workforce And Practice List Sizes** publication. Figures for estimated GP headcount and WTE presented in previous surveys (from 2013 to 2017) have also been scaled to match the NPCCD headcount to provide a comparable trend. Specialist Trainees/Performer Registrars

(medical practitioners in a GP training program) are excluded from this publication in order to be comparable to previous surveys.

For more details on the methodology used, see the **Methodology section** of this report.

Main points

- The GP headcount increased from 4,400 in 2019 to 4,514 in 2022, an increase of just below 3%.
- The estimated GP (excluding Specialist Trainees) WTE decreased from 3,613 in 2019 to 3,494 in 2022, a decrease of 3%.
- Female GPs were more likely to work part-time than male GPs. On average there was 0.71 WTE for every female GP and 0.88 WTE for every male GP.
- Maternity Leave was the biggest source of GP absence (approx. 36,000 estimated sessions in 2021/22), followed by Sick Leave (approx. 27,000 estimated sessions) and Self Isolation (approx. 21,000 estimated sessions).
- The estimated WTE for all nurses in general practice was 1,690 (based on 37 hours or more per week being full time), the same as the previous survey in 2019.
- The largest group of nurses in general practice were General Practice Nurses, accounting for 63% (headcount) and 60% (WTE).
- There were an estimated 479 Health Care Assistants (headcount) and 317 (WTE) at general practices in Scotland in 2022.
- There were an estimated 103 Phlebotomists (headcount) and 59.6 (WTE) at general practices in Scotland in 2022.

Results and commentary

This report summarises the results from the **Data Collection Tool**. A full list of charts and tables are available to view in the accompanying **Interactive Dashboard**.

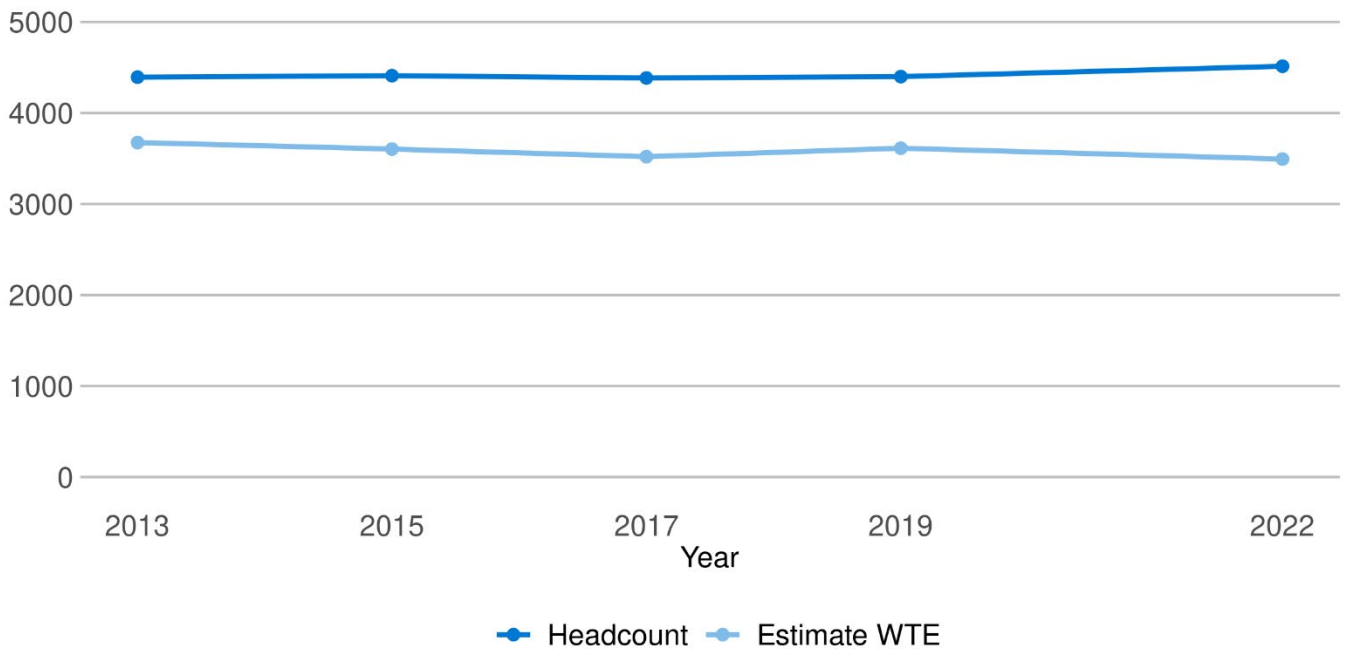
General Practitioners

Headcount and estimated Whole Time Equivalents (WTEs)

This section will explore the GP headcounts and estimated WTEs over time; by NHS Board; by GP designation; and by age group and sex.

Figure 1 charts the GP headcount and estimated WTE over the last five survey years.

Figure 1: Trend in GP headcount[1] and estimated WTE[2,3,4] 2013-2022[5,R]



1. Headcount from the NPCCD and excludes Registrar Trainee GPs.

2. The estimated WTE (in the absence of a 100% survey response rate) was based on scaling the sample headcount from the survey where WTE data were present to match the national headcount from NPCCD. For more details see the **Methodology section**.
3. Previous survey WTEs (up to 2017) have been adjusted according to the NPCCD headcount to ensure a consistent trend. Previously this was estimated using population and differed slightly from the NPCCD (then GPCD) headcount (see Table A3 of the 2017 publication).
4. One WTE is defined in this report as eight weekly contracted GP sessions.
5. As at 31 March for 2022 and **2019**, 31 August for **2017** and **2015**, and 31 January for **2013**.

R. 21/12/2022 at 11.25: Revision of GP headcount and WTE for 2013-2017 to match 2019 GP workforce survey data.

Figure 1 shows that the estimated WTE decreased from 3,613 in 2019 to 3,494 in 2022, a decrease of just over 3%. This followed a minor increase in estimated WTE in 2019 after a decrease between 2013 and 2017.

Table 1 displays the GP headcount and estimated WTE for the 2022 survey year, split by NHS Board.

Table 1: GP headcount[1,2] and estimated WTE[3,4,5], by NHS Board; 31 March 2022

NHS Board	Headcount	Estimated WTE	WTE per headcount	WTE Per 10,000 Patients
Ayrshire & Arran	292	236	0.81	6.1
Borders	105	89	0.85	7.4
Dumfries & Galloway	109	95	0.87	6.1
Fife	269	216	0.80	5.5
Forth Valley	247	184	0.74	5.7
Grampian	441	337	0.76	5.6
Greater Glasgow & Clyde	960	755	0.79	5.6
Highland	373	296	0.79	8.9
Lanarkshire	399	317	0.79	4.6
Lothian	880	635	0.72	6.3
Orkney	51	38	0.74	18.6
Shetland	26	16[6]	0.61[6]	7.3[6]
Tayside	363	281	0.77	6.5
Western Isles	28	22	0.77	8
Scotland	**4,514**	**3,494**	**0.77**	**6**

1. Headcount from the NPCCD and excludes Registrar Trainee GPs.
2. GPs can work in more than one board and so the Scotland total for headcount does not equal the sum of each board.
3. The estimated WTE (in the absence of a 100% survey response rate) was based on scaling the sample headcount from the survey where WTE data were present to match the national headcount from NPCCD. For more details see the **Methodology section**.
4. One WTE is defined in this report as eight weekly contracted GP sessions.
5. The WTE for Scotland has been estimated separately from the WTE for each board, so the Scotland total is slightly different than the sum of the boards WTE.
6. Shetland returned only salaried GPs and therefore the estimated WTE is based on imputed data from the 2019 survey. For more details see the **Methodology section**.

Table 1 shows that three rural NHS Boards (NHS Orkney, NHS Highland and NHS Western Isles) had the highest WTE per population, between 18.6 and 8 WTE per 10,000 patients, while NHS Lanarkshire, NHS Forth Valley, NHS Fife, NHS Greater Glasgow & Clyde, and NHS Grampian reported the lowest, below 6.0 WTE per 10,000 patients.

It also shows that NHS Dumfries & Galloway (0.87 WTE per GP) and NHS Borders (0.85 WTE per GP) had the highest WTE per GP, while NHS Shetland (0.61 WTE per GP) and NHS Lothian (0.72 WTE per GP) had the lowest.

Table 2: GP headcount[1,2] and estimated WTE[3,4,5], by Designation; 31 March 2022

Designation	Headcount	Estimated WTE	WTE per headcount
Performer	3,255	2,656	0.82
Performer Salaried	1221	818[6]	0.67[6]
Performer Retainer	61	27[6]	0.44[6]
All GPs	**4,514**	**3,494**	**0.77**

1. Headcount from the NPCCD and excludes Registrar Trainee GPs.
2. GPs can work in more than one designation, so the Scotland total for headcount does not equal the sum of each designation.
3. The estimated WTE (in the absence of a 100% survey response rate) was derived by multiplying the proportion of the responding practice headcount with session data to the NPCCD headcount with the responding WTE figure.
4. One WTE is defined in this report as eight weekly contracted GP sessions.
5. The WTE for Scotland has been estimated separately from the WTE for each designation, so the Scotland total is slightly different than the sum of the designations WTE.
6. Performer retainer did not return any session data and Performer Salaried could not enter session data due to a fault with the data collection tool. The estimated WTE are based on imputation from the 2019 survey data. For more details see the **Methodology section**.

From Table 2, the GP designation surveyed with the largest headcount was Performer GPs, who had an average of 0.82 WTE per GP.

Table 3: GP headcount[1] and estimated WTE[1,2,3,4], by Sex; 31 March 2022

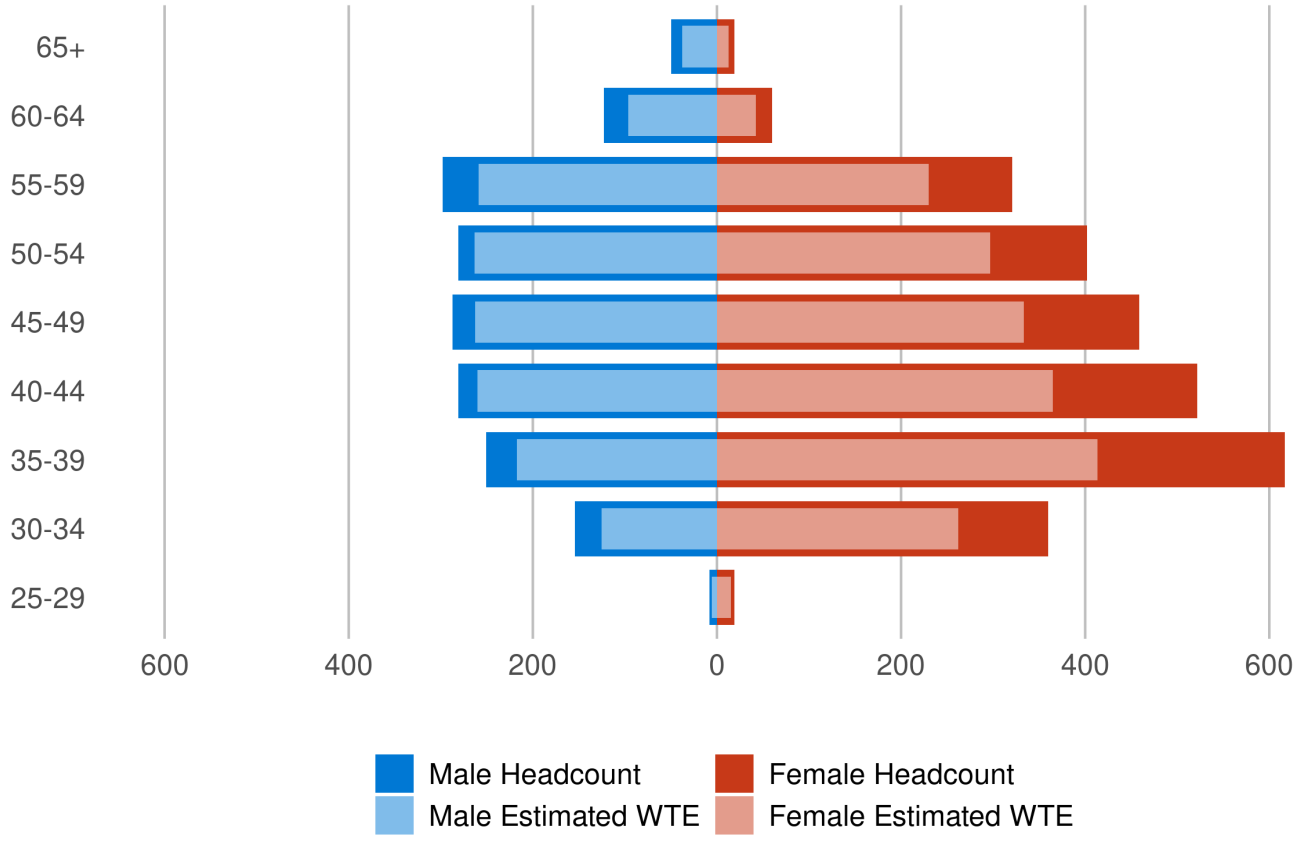
	Headcount	Estimated WTE	WTE per headcount
Male	1,733	1,527	0.88
Female	2,779	1970	0.71
All GPs	**4,514**	**3,494**	**0.77**

1. Headcount from the NPCCD and excludes Registrar Trainee GPs.
2. The estimated WTE (in the absence of a 100% survey response rate) was derived by multiplying the proportion of the responding practice headcount with session data to the NPCCD headcount with the responding WTE figure.
3. One WTE is defined in this report as eight weekly contracted GP sessions.
4. The WTE for Scotland has been estimated separately from the WTE for both males and females, so the Scotland total is slightly different than the sum of the estimated male and female WTE.

Table 3 shows that, overall, there was 0.71 WTE for every female GP and 0.88 WTE for every male GP.

Figure 2 explores the GP headcounts and estimated WTE by age and sex in a population pyramid. While the red shades represent the figures for females and blue for males, the darker shades of these colours represent the headcount, and the lighter shades represent the estimated WTE.

Figure 2: GP headcount[1] and estimated WTE[1,2,3], by Age and Sex[4]; 31 March 2022



1. Headcount from the NPCCD and excludes Registrar Trainee GPs.
2. The estimated WTE (in the absence of a 100% survey response rate) was based on scaling the sample headcount from the survey where WTE data were present to match the national headcount from NPCCD. For more details see the **Methodology section**.
3. One GP WTE is defined in this report as eight weekly contracted GP sessions.
4. Due to missing session data from performer salaried and performer retainer it is possible the data are skewed as most performer salaried are female.

There were more female GPs than male in every age group under 60. Similarly, the difference between headcount and estimated WTE was higher in females than males within every age group, except for those aged 60 and above.

The demographic group with the lowest rate of WTE per GP headcount was female GPs aged 65+ (0.66 WTE per headcount). It is worth noting that there were less than

20 female GPs aged 65+, therefore each WTE would have a large impact on WTE per headcount. The group with the largest headcount were female GPs aged 35-39, and that was also the group with the second lowest WTE per headcount (0.67). Males between the ages of 40 and 54 had the highest rates of WTE per GP headcount, with an average estimate of 0.93 WTE per GP. This suggests that almost all males within this age group are working full time, where full time is defined as 8 sessions per week.

Absences and Use of locum/sessional GP time

This section will explore the use of locum and sessional GP time, along with the reasons for GP absences.

Table 4 shows the percent of responding practices that used a locum GP along with the estimated Locum GP WTEs, displayed by NHS Board. In this report, one locum GP WTE is defined as 416 sessions over 2021/22 – an average of eight sessions per week during the financial year.

Table 4: Estimated Locum/Sessional GP WTE[1,2,3], by NHS Board; 1 April 2021 to 31 March 2022

NHS Board	Percent of Responding Practices Using a Locum GP	Estimated Locum WTE
Ayrshire & Arran	82%	10
Borders	100%	11
Dumfries & Galloway	78%	9
Fife	84%	16
Forth Valley	79%	8
Grampian	88%	34
Greater Glasgow & Clyde	85%	63
Highland	80%	29
Lanarkshire	85%	34
Lothian	97%	53
Orkney	75%	6
Shetland	100%	16
Tayside	93%	18
Western Isles	67%	2
Scotland	**86%**	**292**

1. Locum GP WTE calculated as the total number of locum sessions filled during 2021/22 divided by 416 (the eight sessions that make up a weekly WTE multiplied by the 52 weeks in the financial year).
2. The estimated WTE (in the absence of a 100% survey response rate) was based on scaling the sample headcount from the survey to match the national headcount from NPCCD. For more details see the **Methodology section**.
3. The WTE for Scotland has been estimated separately from the WTE for each board, so the Scotland total is slightly different than the sum of the boards' WTE.

Table 4 shows that, overall, 86% of all responding practices reported the use of a locum GP during 2021/22, with the estimated use of 292 Locum GP WTEs. In two boards (NHS Borders and NHS Shetland), 100% of responding practices reported

using a locum GP, while the figure is lowest for NHS Western Isles (67%) and NHS Orkney (75%).

Further details covering the use of Locum GPs can be found in the **Interactive Dashboard** accompanying this report.

Table 5 shows the estimated number of absent GP sessions by the reason for absence. The number of absent sessions from the responding practices were scaled up to produce estimates based on the proportions of the sample headcount and the headcount from NPCCD. For more details see the **Methodology section**.

Table 5: Number of estimated[1] absent GP sessions, by reason for absence; 1 April 2021 to 31 March 2022

NHS Health board	Sick Leave	Maternity Leave	Parental Leave	Special Leave	Self isolation
Scotland	27,296	35,744	2,422	1,974	20,579

1. The estimated number of absent sessions (in the absence of a 100% survey response rate) was based on scaling the sample headcount from the survey to match the national headcount from NPCCD. For more details see the **Methodology section**.

From Table 5, the largest estimated numbers of absent GP sessions were due to Maternity Leave and Sick leave, with an estimate of 35,744 absent GP sessions due to Maternity Leave, and an estimate of 27,296 absent GP sessions due to Sick Leave. In addition to the types of leave in the 2019 survey, practices reported that 20,579 sessions were absent due to self isolation.

A further breakdown of these data by NHS Board can be found in the **Interactive Dashboard** accompanying this report.

Reported GP vacancies

This section will explore the vacancy rates reported by responding practices.

Table 6 details the percent of responding practices reporting a vacancy, by NHS Board. The vacancy rate, defined as the number of vacancy sessions per 100 total GP sessions, is also shown.

From Table 6, just over one third of the responding GP practices reported a vacancy at their practice during 2021/22. The overall vacancy rate was 8.7 vacant GP sessions for every 100 total GP sessions. The vacancy rate varied by NHS Board. Discounting NHS Shetland's rates which are subject to volatility due to small numbers, the vacancy rate ranged from 5.6 vacancy sessions per 100 GP sessions in NHS Greater Glasgow & Clyde, to 13.7 vacancy sessions per 100 GP session in NHS Lanarkshire.

Table 6: Vacant GP Sessions[1] and Vacancy Rates[1,2], by NHS Board; 1 April 2021 to 31 March 2022

NHS Board	Percent of Responding Practices Reporting a Vacancy	Vacancy Rate[2]
Ayrshire & Arran	45%	8.5
Borders	67%	10.1
Dumfries & Galloway	26%	8.8
Fife	47%	10.7
Forth Valley	30%	6.3
Grampian	52%	12.5
Greater Glasgow & Clyde	21%	5.6
Highland	24%	7.2
Lanarkshire	44%	13.7
Lothian	52%	9.1
Orkney	25%	7.4
Shetland	67%	25.5
Tayside	44%	7.8
Western Isles	33%	11.9
Scotland	**37%**	**8.7**

1. Figures for Island boards may be impacted by small numbers.
2. Vacancy rate is the number of vacancy sessions per 100 total GP sessions.

Nurses, Health Care Assistants, Phlebotomists and Other Staff

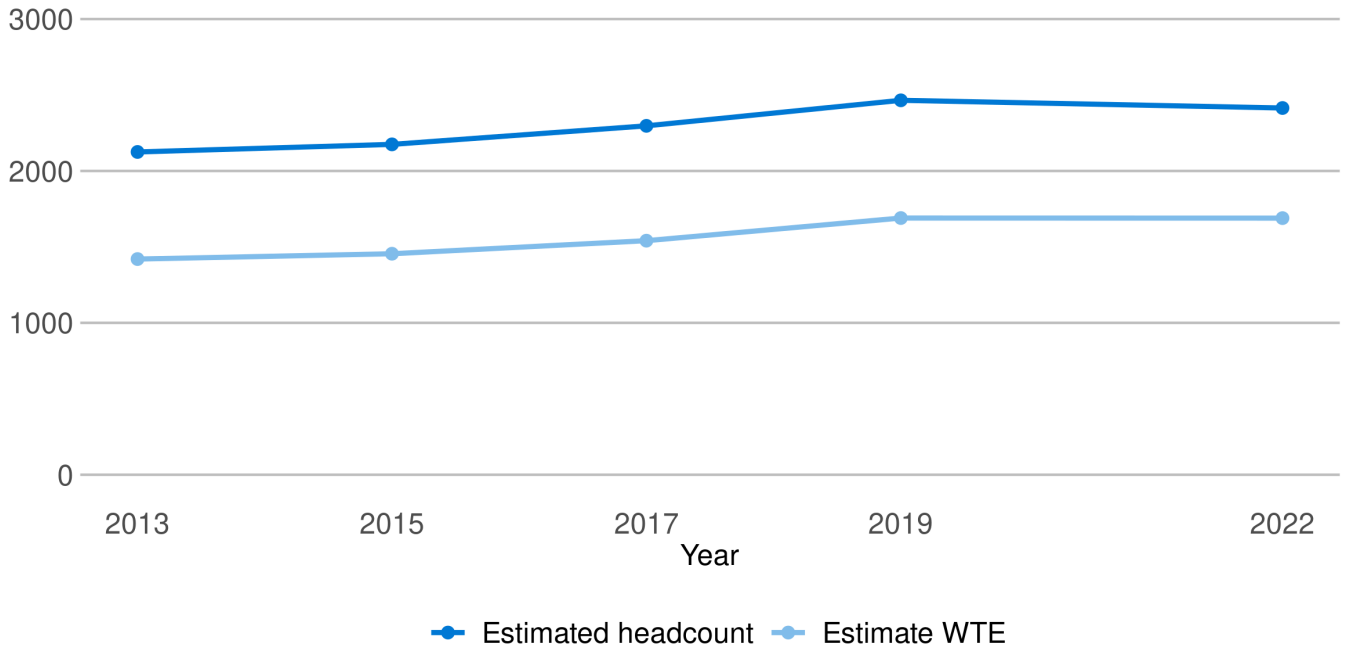
Nurses

Estimated headcount and Whole Time Equivalent (WTE)

This section will explore the estimated nurse headcounts and estimated WTE over time; by NHS Board; by designation; and by age group.

Figure 3 charts the estimated headcount and WTE of nurses working in general practices over the last five survey years.

Figure 3: Trend in nurse estimated[1,2] headcount and WTE; 2013-2022[3]



1. Figures are estimates based on population of practices returning data.
2. One nurse WTE is defined as 37 weekly contracted hours.
3. As at 31 March for 2022 and **2019**, 31 August for **2017** and **2015**, and 31 January for **2013**.

The estimated headcount of registered nurses working in GP practices in Scotland in 2022 was 2,414, a decrease of 51 from the previous survey. The estimated WTE for all nurses was 1,690 (based on 37 hours or more per week being full time), representing the same estimated WTE as the previous survey year. Figure 3 shows that the estimated number of nurses increased by 14% between 2013 and 2022 (from 2,125 to 2,414), and the estimated WTE increased by 19% (from 1,420 to 1,690) in the same period. For the first time since 2013, estimated nurse headcount has decreased between survey years with 2022 showing a reduction by 2% compared to 2019.

Table 7 displays estimated headcount and WTE for nurses by NHS Board.

Table 7: Nurses in general practices - Estimated headcount and WTE[1,2,3] by NHS Board; 31 March 2022

NHS Board	Estimated headcount	Estimated WTE	Estimated WTE per 10,000 patients
Ayrshire & Arran	151	118	3.1
Borders	54	36	3.0
Dumfries & Galloway	85	58	3.7
Fife	158	114	2.9
Forth Valley	107	72	2.2
Grampian	385	260	4.3
Greater Glasgow & Clyde	416	287	2.1
Highland	199	131	3.9
Lanarkshire	252	178	2.6
Lothian	338	238	2.4
Orkney	12	8	4.2
Shetland	0	0	0
Tayside	192	137	3.1
Western Isles	28	26	9.7
Scotland	**2,414**	**1,690**	**2.9**

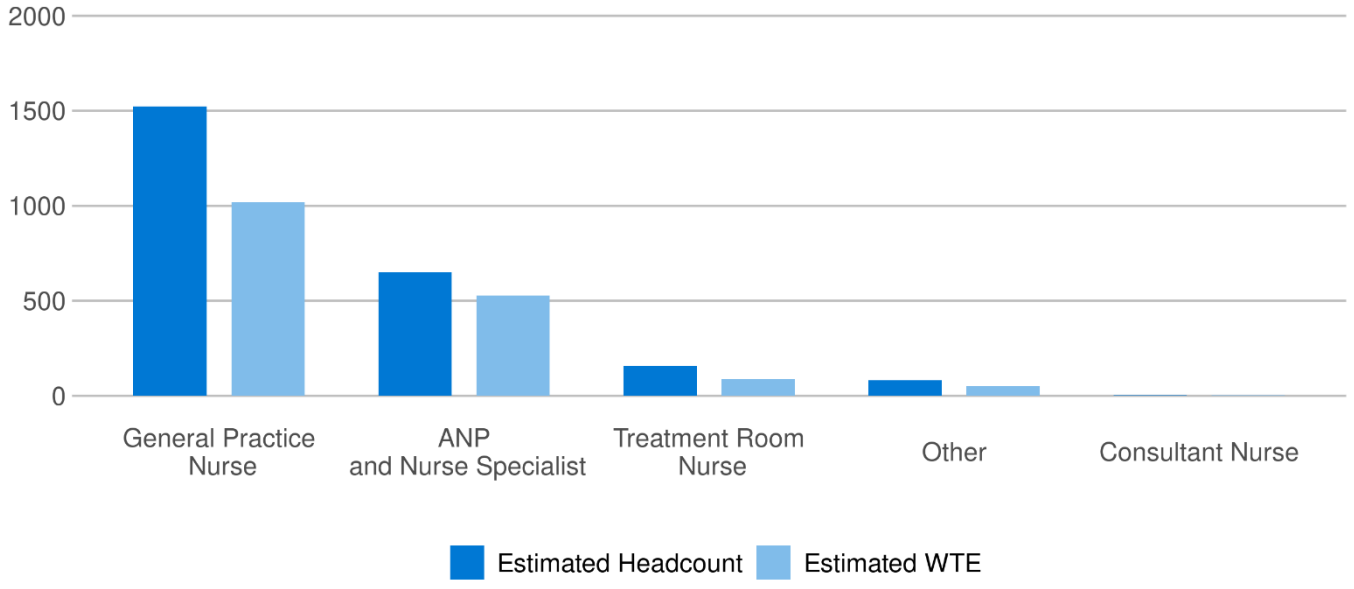
1. Figures are estimates based on population of practices returning data. Small numbers may increase the volatility of estimates for some boards.
2. One nurse WTE is defined as 37 weekly contracted hours.
3. The estimates for Scotland have been calculated separately from the estimates for each board, so the Scotland totals are slightly different than the sum of the boards.

The average rate of estimated WTE per 10,000 patients in Scotland was 2.9. NHS Grampian (4.3) and NHS Highland (3.9) had the highest rates of estimated WTE per 10,000 patients (discounting the Island boards rates which are subject to volatility due to small numbers, and NHS Shetland that did not report any nurses in general

practices). NHS Greater Glasgow & Clyde (2.1) and NHS Forth Valley (2.2) had the lowest rates.

Figure 4 shows the estimated headcount of nurses and estimated WTE for each designation.

Figure 4: Nurses[1] in general practices - Estimated headcount & WTEs[2,3,4], by Designation; 31 March 2022



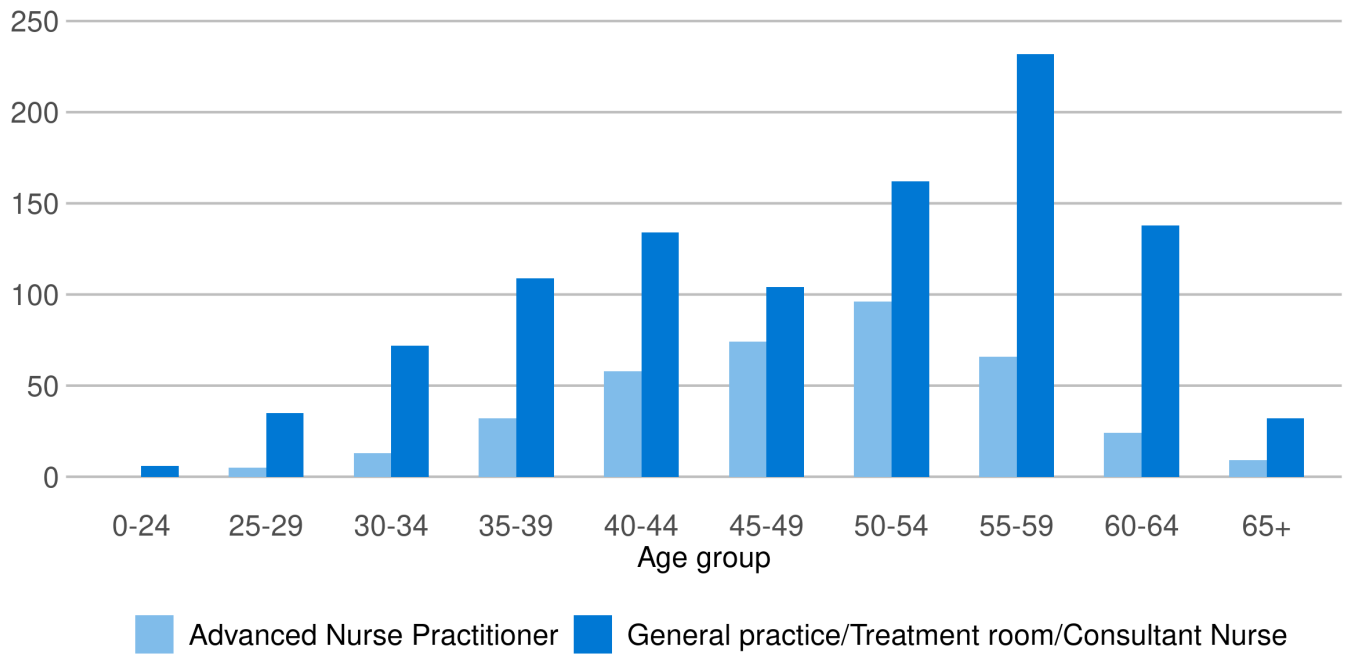
1. General Practice Nurse includes Lead General Practice Nurse, Senior General Practice Nurse, and General Practice Nurse.
2. Figures are estimates based on population of returning practices.
3. One nurse WTE is defined as 37 weekly contracted hours.
4. These figures only include the specified nursing designations and do therefore not aggregate up to total nurses who work in general practices.

The largest group of nurses working at general practices was General Practice Nurses, accounting for 63% of the estimated nurse headcount and 60% of the estimated nurse WTE. The next largest group was Advanced Nurse Practitioners (ANPs) and Nurse Specialists, accounting for 27% of the estimated nurse headcount and 31% of the estimated nurse WTE.

These data, along with further details surrounding nurse designation, can be found within the **Interactive Dashboard** accompanying this report.

Figure 5 illustrates the number of nurses in practices, by age group. Over half (54%) of all nurses registered at practices were aged 50 years and over and

Figure 5: Registered nurses[1] Employed by Responding Practices[2], by Age Group; 31 March 2022



1. General practice nurse includes Lead General Practice Nurse, Senior General Practice Nurse, and General Practice Nurse.
2. One nurse WTE is defined as 37 weekly contracted hours.

Full and part-time working of nurses

With respect to the patterns of working hours of the nurse staff:

- The majority of Advanced Nurse Practitioners (63%) were contracted for 30 hours or more, compared to the General practice nurse group, in which 31% of them were contracted for the same range of hours.
- More than half of General Practice Nurses (57%) and Treatment Room Nurses (66%) were contracted for between 16 and 29 hours, while 32% of Advanced Nurse Practitioners were contracted for these hours.

- A smaller proportion of all designation groups were contracted for fewer than 16 hours; for instance, 5% of Advanced Nurses Practitioners and 10% of the General practice nurses group are in this category.

Nurse vacancies

- Just over one quarter (26%) of responding practices reported that they had nurse vacancies during 2021/22.
- 10% of responding practices reported vacancies for Advanced Nurse Practitioners, while 19% reported vacancies for general practice/treatment room nurses.
- The overall vacancy rate for nurses working out of responding practices was 11.7%, where vacancy rate is vacant nurse hours by total nurse hours.

For more information please see the **Interactive Dashboard** accompanying this report.

Health Care Assistants and Phlebotomists

Headcount and estimated Whole Time Equivalent (WTE)

There were an estimated 479 Health Care Assistants (HCAs) with an estimated WTE of 317 (WTE) working in Scottish general practice in 2022 (as at March 31). This shows a 24% decrease in headcount (estimated 627 in 2019) and a 23% decrease in estimated WTE (410 in 2019) compared with the previous survey.

For phlebotomists, there were an estimated 103 working at general practices in Scotland in 2022, with an estimated WTE of 59.6. This shows a similar estimated headcount as in 2019 (104), but with a 5% higher estimated WTE (54 in 2019) compared with the previous survey.

Please note that these are estimates only - there is likely a higher margin of error due to the smaller numbers involved, and incomplete practice returns (see **Methodology section** *for more information).*

Health Care Assistants & Phlebotomists vacancies

- Only 5% of all responding practices reported that they had HCA or Phlebotomists vacancies.

Further details, including board estimates, can be found in the **Interactive Dashboard** accompanying this report.

Other Staff

Table 8 shows the estimated headcount of other staff working at GP practices in Scotland.

Table 8: Estimated headcount[1] of Other Staff at GP Practices; 31 March 2022

Staff Group	Job Role	Estimated headcount
Non-Clinical	Receptionist	5,618
Non-Clinical	Other[2]	1,177
Non-Clinical	Practice Manager/Manager	1,160
Non-Clinical	Medical Secretary	815
Non-Clinical	Estates and Ancillary	231
Non-Clinical	Apprentice	26
Non-Clinical	Management Partner	16
Non-Clinical	Telephonist	12
Other Direct Patient Care	Pharmacist	65
Other Direct Patient Care	Other Clinical Role[3]	65
Other Direct Patient Care	Dispenser	47

1. Figures are estimates based on population of returning practices.
2. Non-Clinical – Other: Any other administrative staff working within the practice/individuals working across several Admin/Non-Clinical roles.
3. Other Direct Patient Care – Other Clinical Role: Any other professional employed by the GP practice who is directly involved in the treatment and care of patients.

There were an estimated 5,618 receptionists employed in general practices in 2022 (as at March 31). The next largest group was Practice Manager/Manager, with 1,177.

Glossary

Advanced Nurse Practitioner

ANPs are educated at Master's level and are competent to work at advanced level as part of multidisciplinary teams across all clinical settings, dependent on their area of expertise. They are clinical leaders with the freedom and authority to act and accept responsibility and accountability for those actions. The role is characterised by high-level autonomous decision-making, including assessing, diagnosing and treating (including prescribing for) patients with complex multidimensional problems.

Consultant Nurse

Consultant nurses work at a very high level of clinical expertise and have responsibility for contributing to national, as well as local, developments in their recognised area of expertise.

General Practice Nurse

A General Practice Nurse is a qualified and registered nurse involved in almost every aspect of patient care and treatment, for people of all ages.

GP Locum

A GP who provides temporary cover. This can be, for example, to cover whilst a regular GP is on leave, or when a general practice is short staffed.

GP Performer

A GP who has entered into a contract to provide services to patients and is effectively self- employed; usually a practice partner.

GP Performer Retainer

A GP who works part time and can be utilised by a practice as required.

GP Performer Salaried

A GP who is employed by the practice or by the NHS Board on a salaried basis. Also known as a salaried GP.

GP Performer Registrar/Specialist Trainee

A medical practitioner who is being trained in general practice.

GP Whole Time Equivalent

In this report, one GP Whole Time Equivalent is defined as eight contracted weekly GP sessions.

GP session

One session is defined as four hours and ten minutes.

Headcount

A count of the number of staff in post, regardless of whether individuals work full or part-time.

Health Care Assistant

Health Care Assistants work under the guidance of a qualified healthcare professional, usually a nurse. Sometimes known as nursing assistants, nursing auxiliaries or auxiliary nurses.

Lead General Practice Nurse

Has a higher degree of clinical decision-making, autonomy and responsibility than general practice nurses in the clinical environment and has completed the appropriate level of education.

NPCCD

National Primary Care Clinicians Database. A centralised database of GP and general practice details held at NSS but with data maintained by NHS Boards.

Nurse Whole Time Equivalent

In this report, one nurse Whole Time Equivalent is defined as 37 contracted weekly hours.

Phlebotomist

A Phlebotomist collects blood samples from patients.

Senior General Practice Nurse

Senior GPNs will have some advanced skills, will be the expert in general practice nursing team leadership and management and will predominantly see patients who have an established diagnosis.

Treatment Room Nurse

Assist in the care of patients including treatments, preventative care and screening as well as education undertake tasks which are delegated.

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Further information

Further information and data for this publication are available from the **publication page** on our website.

Open data

Data from this publication is available to download from the **interactive dashboard**.

Rate this publication

Let us know what you think about this publication via. the link at the bottom of this **publication page** on the PHS website.

Appendices

Appendix 1 – Background information

The **General practice workforce survey** was last carried out in 2019. It provided aggregate information on general practice workforce, including:

- GPs in practice
- Practice-employed nurses / healthcare support workers / phlebotomists in post
- GP and nurse / healthcare support worker / phlebotomist vacancies and recruitment to posts
- Temporary cover for sessions / hours
- Other staff working out of or based at GP practice premises

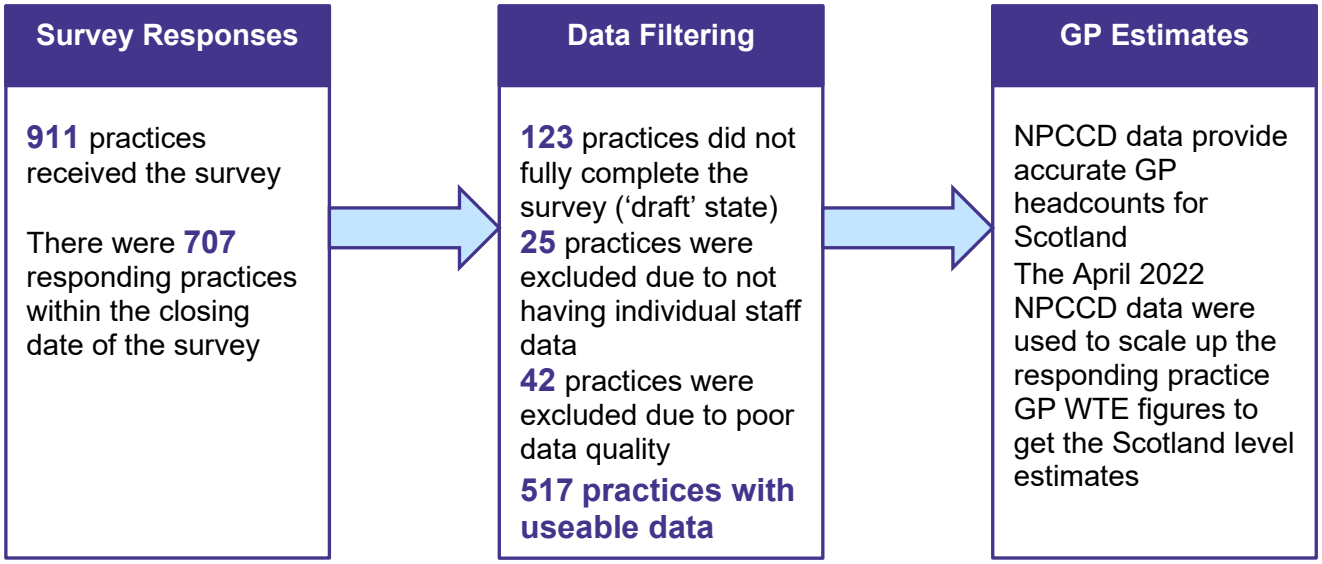
The data was collected using the same **GP Data Collection Tool** as used for the **2019 survey,** which collected data on the GP and nurse workforce in general practice. It also collected aggregate finance information on the income and expense of running a general practice, which is being analysed internally by the Scottish Government.

Information on the Primary Care out of hours workforce will be published separately in early 2023.

For the in-hours survey, the questions asked in 2019 were also asked in 2022. In addition, questions were asked concerning absences due to covid as well as a section on Multi Disciplinary Teams (MDT). Analysis on the MDT was not included in the report due to poor data quality and data completeness.

Further background information and reference files for practices are available on the **PHS Website.**

Appendix 2 – Methodology



Data

There were 707 responding practices overall. Of these, 123 did not fully complete the survey, 25 practices did not return individual staff data and 42 practices were excluded due to poor data quality. This left 517 practices with usable survey response data.

The data quality was determined by calculating the mean error rate for empty and duplicate rows in the data. All practices that exceeded the error rates were excluded.

Instead of collecting National Insurance (NI) numbers as unique identifiers, as was done in the 2019 survey, professional registration numbers for the General Medical Council (GMC) and Nursing Midwifery Council (NMC) were collected. The change in unique identifier data was due to data completeness and data quality issues in 2019, with the NI numbers field resulting in 311 practices' data being removed from the data.

On receipt of the 2022 data, the data quality and completeness of the professional registration number fields were not suitable for using as unique identifiers. In its place, a new unique key for each individual was created using sufficiently complete fields available in the data. This method also allowed a unique key to be created for practice staff who were non-clinical staff; practice manages, receptionists, telephonists, and others who do not have professional registration numbers.

GP WTE Estimates

While examining the data, the analysis team found a significant data deficit for the salaried and retainer GP cohort where no data was received for the number of sessions or hours worked. After discussing several options to mitigate the issue, a new methodology was developed to impute these values. Considerations were taken to mitigate the risk of providing a skewed estimate for the estimated GP WTE given that the salaried GP cohort is mainly women of childbearing age who are more likely to work part time compared to their male counterparts. Thus, excluding this cohort would potentially create an estimate for all GPs WTE which would not consider this large group contributing to the total figure.

The data from the 2019 survey was re-analysed to get the WTE/headcount ratio for each GP designation, age group and gender, and then imputed into the raw data from 2022 where session data were missing due to a known issue. The ratio was used to re-calculate the headcount factors to produce the WTE and then these factors were applied to the 2022 data with imputed figures to create the final estimates.

As an example, for a female salaried GP aged 30-34, instead of recording “NA” sessions per week in the 2022 survey, the imputed data would be calculated by taking the cohort WTE/headcount (2019) * 8 (full time weekly session for GPs).

Once these factors had been considered the headcount scaling factors where then calculated.

Due to some data entries missing session and hours worked data, two separate headcount factors were used. One for headcount that included all entries in the survey data, and one for WTE that included all entries with the relevant session and hours worked data.

Scaling factors for calculating the estimated GP headcount and WTEs were calculated as follows:

$$Scale\ Factor = \ NPCCD\ GP\ Headcount/Responding\ Practice\ GP\ Headcount$$

This scaling factor was calculated for each NHS Board, GP Designation, and each age group and sex, allowing more accurate estimates to be calculated when disaggregating by these groups. The estimates from previous surveys presented in this report (2013, 2015, 2017) have been recalculated using the NPCCD scaling factor to ensure consistent methodology.

Nurse headcount and WTE Estimates

Nurse headcount and WTE estimates were calculated based on population share. Scaling factors were calculated in the following way:

$$Scale\ Factor = \ Total\ List\ Size / Responding\ Practice\ List\ Size$$

The nurse scale factor was also calculated for each NHS Board, GP designation, age group and sex.

Appendix 3 – Publication metadata

Publication title

General Practice Workforce Survey 2022

Description

WTE of GPs, WTE of nurses and headcount of nurses in general practices in Scotland in 2022.

Theme

Health and Social Care

Topic

General Practice

Format

PDF Report and R-Shiny dashboard

Data source(s)

GP Data Collection Tool

Date that data are acquired

2022

Release date

29 November 2022

Frequency

Tbc.

Timeframe of data and timeliness

Data as at 31 March 2022.

Continuity of data

Continuous

Revisions statement

A minor error was uncovered with the GP headcount and WTE for 2013-2017 and

this has now been corrected to match the 2019 publication. The error did not have an impact on any calculations or interpretations of the data. The summary and dashboard have also been corrected.

Revisions relevant to this publication

Footnote for Figure 1 and plot in summary: 21/12/2022 at 11.25: Revision of GP headcount and WTE for 2013-2017 to match 2019 GP workforce survey data.

Second main point in summary in revisions statement: The second main point has been revised to make the interpretation of the data clearer.

Footnote for plot GP headcount and estimated WTE, and Data Explorer table in dashboard: Revision on 21/12/2022 at 11:25. A minor error was uncovered with the GP headcount and WTE for 2013-2017 and this has now been corrected to match the 2019 publication. The error did not have an impact on any calculations or interpretations of the data.

Footnote for plot GP headcount and estimated WTE per sex, and Data Explorer table in dashboard: Revision on 21/12/2022 at 11:25. Data entry error, the value of Male GP estimated WTE for 2015 is 1826, not 1829 as previously stated.

Concepts and definitions

See Glossary and Methodology sections of this report for further information.

Relevance and key uses of the statistics

Some 90% interactions with the health service are made through primary care. This publication makes information available to the Scottish Government and NHS Boards for planning and provision of primary care services.

Accuracy

Figures presented from this survey are estimates based on a sample of 517 practices in Scotland. Every effort has been taken to make the estimates as accurate as possible, but they are not exact figures. For more information see the Methodology section of this report.

Completeness

Estimates are based on 517 practices in Scotland out of more than 900.

Comparability

Scottish GP workforce information is broadly comparable with GP workforce information for the other UK countries. However, there are some variations in how the individual GP designations/contract types are described, and care must be taken to ensure that like for like groups are being compared (e.g., whether GP trainees or retainers are or are not included in each group being compared).

Some Scotland-level trends are presented comparing data from 2022 to previous National Primary Care Workforce Surveys and every effort has been made to make figures for 2022 comparable to 2019 and before. Due to the different method of collecting data in 2019, direct comparisons to data from 2019 to that released in the 2017 National Primary Care Workforce Survey should be made with caution, especially at NHS Board level.

Accessibility

It is the policy of Public Health Scotland to make its web sites and products accessible according to published guidelines. More information on accessibility can be found on the **PHS website**.

Coherence and clarity

An Interactive Dashboard is available to view on the PHS Website.

Value type and unit of measurement

Numbers and percentages.

Disclosure

The **PHS protocol on Statistical Disclosure Protocol** is followed.

Official Statistics designation

Experimental Statistics

UK Statistics Authority Assessment

N/A

Last published

26 October 2021

Next published

TBC.

Date of first publication

26 October 2021 with **Data Collection Tool**

24 September 2013 with **National Primary Care Workforce Survey**

Help email

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Date form completed

02 September 2022

Appendix 4 – Early access details

Pre-Release Access

Under terms of the "Pre-Release Access to Official Statistics (Scotland) Order 2008", PHS is obliged to publish information on those receiving Pre-Release Access ("Pre-Release Access" refers to statistics in their final form prior to publication). The standard maximum Pre-Release Access is five working days. Shown below are details of those receiving standard Pre-Release Access.

Standard Pre-Release Access:

Scottish Government Health Department

NHS Boards

Early Access for Management Information

These statistics will also have been made available to those who needed access to ‘management information’, ie as part of the delivery of health and care:

N/A

Early Access for Quality Assurance

These statistics will also have been made available to those who needed access to help quality assure the publication:

British Medical Association

Appendix 5 – PHS and Official Statistics

About Public Health Scotland (PHS)

PHS is a knowledge-based and intelligence driven organisation with a critical reliance on data and information to enable it to be an independent voice for the public's health, leading collaboratively and effectively across the Scottish public health system, accountable at local and national levels, and providing leadership and focus for achieving better health and wellbeing outcomes for the population. Our statistics comply with the **Code of Practice for Statistics** in terms of trustworthiness, high quality and public value. This also means that we keep data secure at all stages, through collection, processing, analysis and output production, and adhere to the '**five safes**'.